PREOPERATIVE DIAGNOSIS: Slowly growing, painless 2.5 cm mobile mass in the tail of the right parotid gland; fine-needle aspiration consistent with benign pleomorphic adenoma.

POSTOPERATIVE DIAGNOSIS: Pleomorphic adenoma, superficial lobe of the right parotid gland.

PROCEDURE PERFORMED: Right superficial parotidectomy with facial nerve dissection and continuous intraoperative neuro-monitoring.

OPERATIVE TECHNIQUE: Under general anesthesia without neuromuscular blockade to permit facial nerve monitoring (NIM-Response 3.0), the patient was positioned supine with the head turned to the left. A modified Blair incision was marked starting in the preauricular crease, curving around the lobule, and extending into a natural cervical skin crease 2 cm below the angle of the mandible.

The skin flap was elevated anteriorly over the parotid fascia in the superficial musculoaponeurotic system (SMAS) plane. The anterior border of the sternocleidomastoid muscle was mobilized, and the posterior belly of the digastric muscle was identified. The cartilaginous tragal pointer was dissected. Using the tragal pointer (pointing 1 cm inferior and deep to the main trunk) and the tympanomastoid suture line as anatomical landmarks, the main trunk of the facial nerve was identified exiting the stylomastoid foramen. Nerve stimulator confirmed brisk electrical conduction at 0.5 mA with corresponding right hemifacial twitching.

The main trunk was traced into the parotid parenchyma to its primary bifurcation (pes anserinus). Meticulous dissection was carried out along the temporofacial and cervicofacial divisions, identifying and preserving all five peripheral branches (temporal, zygomatic, buccal, marginal mandibular, and cervical). The superficial parotid lobe containing the 2.5 cm encapsulated mass was dissected cleanly off the underlying branches without tumor capsule violation.

The superficial parotidectomy specimen was excised in continuity. Final stimulation of each facial nerve branch verified 100% preservation of electrical and clinical function. A 10-French Jackson-Pratt closed suction drain was placed, and the wound closed in layers with 3-0 Vicryl and 4-0 Monocryl.